

Scout — Practice Roles & Permissions

Draft for approval · Vet At Your Door · practice-level roles only (platform / multi-tenant roles excluded)

What this is. A proposed default role set for a Scout practice, and who can do what across the whole product. It is modeled on the conventional veterinary PIMS baseline you supplied, then rewritten against Scout's actual screens and actions — mobile-practice specifics (routing, zones, room loader, forward booking, autoship, post-visit membership re-rating) have no equivalent in a conventional PIMS, so those rows are new.

What I need from you. Approve or edit the role list in Section 1, the retirements in Section 2, the matrix in Section 4, and the open decisions in Section 7. Nothing is implemented yet.

1. Proposed practice role set

Ten roles. Seven already exist in Scout's role catalog, three are new. A person can hold more than one role (for example Lead Technician + Inventory Manager); permissions are additive.

#	Role	Code	Slug	Who it is for / what it owns	Per branch?	Status
1	Practice Admin	PA	practice_admin	Owner or designated administrator. Full practice control including staff accounts, roles, integrations configuration, and all money. Does not include platform/multi-tenant control.	No	RENAME
2	Practice Manager	PM	practice_manager	Day-to-day operations and finance. Everything except creating admin-level logins and changing integrations/system configuration.	Yes	KEEP
3	Veterinarian (DVM)	DVM	veterinarian	Licensed provider. Sole authority to sign/lock a medical record, write an addendum, and approve a prescription. Owns clinical content; limited on pricing and refunds.	No	NEW
4	Lead Technician	LT	lead_technician	Senior technician. Technician scope plus team task reassignment, lab result review, stock movement, and mail-order approval.	No	KEEP
5	Technician	TE	technician	Vitals, exam findings, treatments, charge capture, vaccine/lot documentation, van stock. Cannot sign a record or approve an Rx.	No	KEEP
6	Lead Client Liaison	LC	lead_client_liaison	Senior CL. CL scope plus seat assignment, merges, membership changes, deposits, and outreach reporting.	No	KEEP
7	Client Liaison	CL	client_liaison	Phones, email, outreach, booking, holds, waitlist, forward booking, payments, membership enrollment.	No	KEEP
8	Inventory Manager	IM	inventory_manager	Stock, lots, counts, suppliers, purchase orders, cost, online store catalog, mail-order fulfillment. Cost yes; sell price needs PM.	Yes	KEEP
9	Billing & AR	BA	billing_ar	Collections, statements, pay links, refunds, deposits, reconciliation, AR reporting. No clinical editing, no scheduling.	No	NEW
10	Observer (read-only)	OB	observer	New hires in training, shadowing students, outside auditors, bookkeepers. Can see records and reports; can change nothing.	No	NEW

Why these three are new

- **Veterinarian (DVM) — the significant gap.** Scout's role catalog today has no provider role at all. Record signing, Rx approval, and provider-scoped production reporting have no role to attach to. This is the one addition I would not skip.

- **Billing & AR** — refunds, voids, deposits, and reconciliation are currently only reachable by making someone a full admin. A narrow role lets a bookkeeper work without clinical or configuration access. *Optional — see Decision 2 if you would rather fold this into Lead Client Liaison.*
- **Observer** — there is no way today to give someone look-only access. Useful for onboarding and for your accountant. *Optional — see Decision 5.*

2. Roles to retire

These exist in the database today but should not appear in a practice's role picker.

Role	Source	Why it should go	Where its people go
Superadmin RETIRE	Scout seed catalog	Duplicates the platform login role of the same name. A practice-level job role called "Superadmin" invites someone to grant practice staff what reads as platform control. Platform access should never be assignable from practice settings.	Stays a login-account attribute, managed by Vet At Your Door, not by the practice.
Admin RENAME	Scout seed catalog	Name collides with the login role <code>admin</code> , so "is this person an admin?" has two different answers today.	Becomes Practice Admin .
Receptionist RETIRE	eVet import	Scout's equivalent is Client Liaison, which is the term the practice actually uses.	Client Liaison.
BusinessOwner, BusinessManager RETIRE	eVet import	Overlap entirely with Practice Admin / Practice Manager.	Practice Admin or Practice Manager.
SystemAdmin, UserManager, ApiManager RETIRE	eVet import	Platform-level concepts imported from the old PIMS; not practice job roles.	Practice Admin, or platform side.
TechnicianAssistant, KennelWorker, Groomer, ShiftLeader RETIRE	eVet import	Brick-and-mortar hospital roles. A mobile practice has no kennel, no grooming, and no shifts in that sense.	Technician, or no role.
Kiosk, Guest, Client RETIRE	eVet import	Not staff. "Client" is a portal account, not an employee role.	Not staff roles.
BillableItemsManager, ReferralSystemManager, LimitedAssignedProvider RETIRE	eVet import	No corresponding feature in Scout.	Practice Manager or Inventory Manager.
GroupManager, GroupOwner, Enterprise, EVet* RETIRE	eVet import	Vendor and multi-group artifacts from the old system.	None.

Retire = hide from the Scout role picker and stop assigning. Existing historical assignments stay in the database for audit; staff get remapped to the column on the right.

3. How to read the matrix

● **Full** — can do it **text Limited** — can do it only as described in the cell ○ **None** — cannot do it, control is hidden

Role codes: PA Practice Admin · PM Practice Manager · DVM Veterinarian · LT Lead Technician · TE Technician · LC Lead Client Liaison · CL Client Liaison · IM Inventory Manager · BA Billing & AR · OB Observer

"Reason code" means the action is allowed but Scout requires a selected reason and records it to the audit report. "Capped %" means a discount above the practice cap routes to Practice Manager approval. "Own" means limited to that person's own visits, entries, or schedule.

4. Permission matrix

Permission	PA	PM	DVM	LT	TE	LC	CL	IM	BA	OB
A • System, users & roles										
Create / edit staff logins, reset staff passwords	●	Non-admin logins only	○	○	○	○	○	○	○	○
Assign practice roles to staff	●	●	○	○	○	○	○	○	○	○
Create / edit the role definitions themselves	●	○	○	○	○	○	○	○	○	○
Manual-booking permissions per role	●	●	○	○	○	○	○	○	○	○
Deactivate or delete an employee record	●	●	○	○	○	○	○	○	○	○
Platform: practices, domains, feature flags, secrets	○	○	○	○	○	○	○	○	○	○
View audit / usage log	●	●	○	○	○	○	○	○	Financial entries	○
B • Practice configuration										
Letterhead, branches & locations	●	●	○	○	○	○	○	Locations only	○	○
Appointment types, durations, arrival windows	●	●	View	○	○	View	View	○	○	○
Practice booking goals	●	●	○	○	○	View	○	○	○	○
Staff schedules, zones, depots, offices	●	●	Own, view only	○	○	View	View	○	○	○
Staff photo, bio & provider signature	●	●	Own	Own	Own	Own	Own	Own	Own	○
CL seat assignment (phones / outreach / email)	●	●	○	○	○	●	View	○	○	○
Automatic task rules	●	●	○	○	○	○	○	○	○	○
Room Loader configuration — offers, pricing, pitch	●	●	Lab panels & clinical copy	○	○	View	○	○	○	○
Email & text templates, reminders, forms, chat hours	●	●	Clinical templates	○	○	●	○	○	○	○
Gmail shared-mailbox permissions	●	●	○	○	○	○	○	○	○	○
Euthanasia consent defaults, aftercare offerings	●	●	●	○	○	View	○	○	○	○
Outside hospitals directory	●	●	●	●	○	●	●	○	○	○
Patient statuses	●	●	○	○	○	○	○	○	○	○
Client discount statuses / tiers	●	●	○	○	○	○	○	○	View	○
Payment types & discount categories	●	●	○	○	○	○	○	○	View	○
Deposit bank accounts	●	●	○	○	○	○	○	○	View	○
Sales tax levels	●	●	○	○	○	○	○	○	○	○
Mail shipping types	●	●	○	○	○	○	○	●	○	○
Waste reasons & disposal methods	●	●	○	○	○	○	○	●	○	○
In-house lab form templates & analytes	●	●	●	●	○	○	○	○	○	○

[illegible]

Permission	PA	PM	DVM	LT	TE	LC	CL	IM	BA	OB
Client communication — text, email, log the call	●	●	●	●	●	●	●	Mail order only	Billing only	○
Household AI chart chat & summaries	●	●	●	●	●	●	●	○	○	View
Pet snapshots — download & use for social	●	●	○	○	○	●	Consented only	○	○	○
E • Medical records & visits										
View the medical record	●	●	●	●	●	●	●	Rx & dispensing	Billing-relevant	●
Create / edit SOAP while unlocked	○	○	●	Vitals, exam, treatments	Vitals, exam, treatments	○	○	○	○	○
Medical note (non-SOAP) entries	Administrative	Administrative	●	●	Own entries	Call notes	Call notes	○	○	○
AI Scribe — record, transcribe, structure	○	○	●	●	●	○	○	○	○	○
Edit the provider scribe prompt	●	○	Own prompt	○	○	○	○	○	○	○
Delete a scribe transcript	●	○	Own visit	○	○	○	○	○	○	○
Sign & lock the record / finalize a note	○	○	●	○	○	○	○	○	○	○
Write an addendum after lock	○	○	●	○	○	○	○	○	○	○
Client recap email — send or skip	●	●	●	●	●	●	●	○	○	○
Callbacks — create, complete, delete	●	●	●	●	Create & complete	●	Create & complete	○	○	○
Reminders — decline or delete on a chart	●	●	●	●	Decline only	●	Decline only	○	○	○
Chronic problems & chronic medications	○	○	●	Record, not start/stop	○	○	○	○	○	○
Upload outside records to the chart	●	●	●	●	●	●	●	○	○	○
Print certificates — rabies, vaccine, health, death	●	●	●	●	●	●	●	○	○	○
Send a client form for signature	●	●	●	●	●	●	●	○	○	○
Enter in-house lab results	○	○	●	●	●	○	○	○	○	○
Review / release lab results to the record	○	○	●	Flag for DVM	○	○	○	○	○	○
Euthanasia — send consent & estimate	●	●	●	●	●	●	●	○	○	○
Room Loader — send, reconcile, confirm matches	●	●	●	●	●	●	●	○	○	○
F • Prescriptions, vaccines & controlled substances										
Enter Rx directions / sig	○	○	●	Draft for approval	Draft for approval	○	○	○	○	○
Approve a prescription	○	○	●	○	○	○	○	○	○	○
Authorize a refill	○	○	●	○	○	○	○	○	○	○
Vaccine lot & dose documentation	●	●	●	●	●	○	○	Lot data	○	○
Flag an item as a controlled substance	●	●	View	○	○	○	○	●	○	○
Controlled-substance count & reconciliation	●	●	Dispensing log	Count with a DVM	○	○	○	●	○	○

Permission	PA	PM	DVM	LT	TE	LC	CL	IM	BA	OB
G · Tasks										
View, claim, complete tasks assigned to me	●	●	●	●	●	●	●	●	●	View
Create a task and assign it to someone else	●	●	●	●	●	●	●	●	●	○
Reassign / bulk reassign tasks	●	●	Own team	●	○	●	○	Inventory tasks	Billing tasks	○
Remove a task (reason code)	●	●	Own	Own team	○	●	○	Inventory tasks	Billing tasks	○
Mail-order task — approve or deny fulfillment	●	●	●	●	○	○	○	●	○	○
Run automatic task rules on demand	●	●	○	○	○	○	○	○	○	○
H · Estimates & invoicing										
View invoices & household ledger	●	●	●	●	●	●	●	Stock lines	●	View
Create an estimate	●	●	●	●	●	●	From price list	○	●	○
Email an estimate / authorize a card hold	●	●	●	●	●	●	●	○	●	○
Add line items to an open invoice	●	●	●	●	Charge capture	●	From price list	○	●	○
Edit or remove line items	●	●	Own invoices	Own entries	Own entries	●	○	○	●	○
Change a price on the invoice	●	●	Reason code	Reason code	Reason code	Reason code	○	○	Reason code	○
Apply a discount	●	●	Capped % + reason	Capped % + reason	Capped % + reason	Capped % + reason	Coded only	○	Capped % + reason	○
Choose membership coverage on a line	●	●	●	●	●	●	●	○	●	○
Set inventory branch / lot on a line	●	●	●	●	●	○	○	●	○	○
Close / finalize an invoice	●	●	●	●	○	●	●	○	●	○
Reopen / unlock a finalized invoice	●	●	○	○	○	Same day	○	○	●	○
Void an invoice or delete a draft	●	●	○	○	○	Same day	○	○	●	○
Process a return / issue account credit	●	●	○	○	○	●	○	○	●	○
Adopt / edit a legacy eVet invoice	●	●	○	○	○	Same day	○	○	●	○
Email invoice, receipt or household statement	●	●	●	●	●	●	●	○	●	○
I · Payments, refunds & deposits										
Take a payment — cash, check, card, terminal	●	●	●	●	●	●	●	○	●	○
Charge a card on file	●	●	●	●	●	●	●	○	●	○
Send a pay link by text or email	●	●	●	●	●	●	●	○	●	○
Void a payment / tender	●	●	Own visit, same day	○	○	●	○	○	●	○
Refund a card payment (reason code)	●	●	●	●	●	●	●	○	●	○
Post a manual account credit / adjustment	●	●	○	○	○	●	○	○	●	○
Create a bank deposit	●	●	○	○	○	●	○	○	●	○

Permission	PA	PM	DVM	LT	TE	LC	CL	IM	BA	OB
Void a deposit	●	●	○	○	○	○	○	○	With PM approval	○
Stripe / Square reconciliation	●	●	○	○	○	○	○	○	●	○
J • Memberships										
Enroll a patient in a plan	●	●	●	●	●	●	●	○	●	○
Post-visit signup — re-rate & refund the visit	●	●	●	●	●	●	●	○	●	○
Override the post-visit window (reason code)	●	●	○	○	○	●	○	○	With PM approval	○
Change plan / carry over usage	●	●	Clinical benefit swap	○	○	●	○	○	●	○
Add or edit a per-patient benefit override	●	●	Clinical benefit	○	○	●	○	○	Billing benefit	○
Cancel a membership	●	●	○	○	○	●	○	○	●	○
Create / edit membership plans & benefits	●	●	○	○	○	○	○	○	○	○
Apply plan changes to existing members	●	With owner approval	○	○	○	○	○	○	○	○
Archive a plan	●	●	○	○	○	○	○	○	○	○
K • Catalog & pricing										
View the price list	●	●	●	●	●	●	●	●	●	●
Add items, procedures, labs	●	●	Request only	○	○	○	○	●	○	○
Edit cost	●	●	○	○	○	○	○	●	○	○
Edit sell price, service fee, minimum price	●	●	○	○	○	○	○	Needs PM approval	○	○
Bulk price change	●	●	○	○	○	○	○	○	○	○
Quantity price breaks	●	●	○	○	○	○	○	●	○	○
Tax level on an item	●	●	○	○	○	○	○	○	○	○
Clinical fields — dispensable, vaccine, controlled	●	●	●	View	○	○	○	●	○	○
Tagalong / auto-add rules	●	●	Clinical rules	○	○	○	○	Stock rules	○	○
Catalog reminder rules	●	●	●	○	○	●	○	○	○	○
Bundles / packages	●	●	○	○	○	○	○	○	○	○
Archive / restore a catalog item	●	●	○	○	○	○	○	●	○	○
L • Inventory & pharmacy										
View stock on hand & lots	●	●	●	●	●	View	View	●	○	View
Receive a shipment, enter vendor invoice & cost	●	●	○	Receive, not cost	○	○	○	●	○	○
Initial entry / seeded lot corrections	●	●	○	○	○	○	○	●	○	○
Move stock between branches & vehicles	●	●	Own vehicle	●	Own vehicle	○	○	●	○	○

Permission	PA	PM	DVM	LT	TE	LC	CL	IM	BA	OB
Waste / adjust with reason	●	●	Own vehicle	●	Own vehicle	○	○	●	○	○
Weekly & full counts — enter and submit	●	●	○	●	●	○	○	●	○	○
Count report & variance review	●	●	○	View	○	○	○	●	○	○
Par levels & reorder points	●	●	○	Suggest	○	○	○	●	○	○
Suppliers	●	●	○	○	○	○	○	●	○	○
Cost reviews — apply or dismiss	●	●	○	○	○	○	○	●	○	○
Expiring report	●	●	View	●	View	○	○	●	○	○
Order list / purchase orders	●	●	Request	Request	Request	○	○	●	○	○
Transfer & fill lists	●	●	Own vehicle	●	●	○	○	●	○	○
Inventory activity log	●	●	View	●	View	○	○	●	○	View
M · Online store & mail order										
Store listings, variations, product content	●	●	○	○	○	○	○	●	○	○
Store categories	●	●	○	○	○	○	○	●	○	○
Catalog / Ecwid import	●	●	○	○	○	○	○	●	○	○
Autoship settings (per item and in bulk)	●	●	○	○	○	○	○	●	○	○
Coupons	●	●	○	○	○	Issue existing	○	○	○	○
Client subscriptions — edit, pause, cancel	●	●	○	○	○	●	Client's own	●	●	○
Abandoned cart — resend	●	●	○	○	○	●	●	●	○	○
Mail order queue — fill, ship, notify	●	●	○	●	●	○	○	●	○	○
N · Reports & analytics										
Payments & collections	●	●	○	○	○	AR aging	○	○	●	View
Production (VSD) — all providers	●	●	Own only	○	○	○	○	○	○	○
VSD vs payments match	●	●	○	○	○	○	○	○	●	○
Projected revenue	●	●	○	○	○	○	○	○	View	○
Square reconciliation	●	●	○	○	○	○	○	○	●	○
Discount / void / refund audit report	●	●	○	○	○	○	○	○	●	○
Appointments, booking calendar, cancellations	●	●	Own only	●	Own only	●	Own only	○	○	View
Patient dormancy & lapsed clients	●	●	Own patients	●	○	●	●	○	○	View
Email tracking, OpenPhone calls, CL performance	●	●	○	○	○	●	Own only	○	○	○
Time spent / route efficiency	●	●	Own only	●	○	●	○	○	○	View
Membership purchases	●	●	○	○	○	●	Own only	○	●	View
Inventory totals & valuation	●	●	○	○	○	○	○	●	View	View

Permission	PA	PM	DVM	LT	TE	LC	CL	IM	BA	OB
Room loader declines, survey results, call coaching	●	●	Own visits	●	○	●	○	○	○	○
Export / download data in bulk	●	Operational reports	○	○	○	○	○	Inventory	Financial	○

5. Conventions & guardrails behind the matrix

1. **Only a DVM signs.** Sign & lock, addendum after lock, Rx approval, and refill authorization are DVM-only, with no admin override. An administrator who is also licensed holds the DVM role in addition to Practice Admin. This is the one place where "admin can do everything" is deliberately false.
2. **Price changes and discounts are capped, not blocked.** Providers, technicians, and liaisons can discount within a practice cap with a mandatory reason code; anything larger routes to Practice Manager. The discount/void/refund audit report is the real control — a hard block just pushes people into giving away free line items instead.
3. **Refunds stay open to client-facing roles.** The usual industry guardrail (same-day void only, or a dollar ceiling) does not survive your post-visit membership flow, where a client enrolls after the appointment and the visit is re-rated retroactively. Refunds therefore require a reason code and show on the audit report rather than being restricted by role.
4. **Removing a line item is an audit event.** Technicians and providers can remove their own entries only. Every removal appears on the void/discount report, because a silent removal is a missed charge.
5. **Lead versus non-lead.** The difference is consistently: team task reassignment, record merges, same-day financial reversals, and team reporting. Everything clinical or client-facing is identical.
6. **"Own vehicle" for a mobile practice.** Providers and technicians adjust and move stock on the van they are assigned to; branch and depot stock stays with Inventory Manager. Inventory Manager and Practice Manager are branch-scoped roles, so a person can hold them for one location only.
7. **Inventory Manager owns cost, not sell price.** They set cost, receive, count, and reorder; manual sell-price overrides and bulk price changes stay with Practice Manager and above. This is the common split and it is what your sample assumed.
8. **Observer is enforced by hiding controls, not by disabling them.** An Observer should never see a Save button they cannot press.
9. **Roles stack.** Someone who is both Lead Technician and Inventory Manager gets the union of both. No role subtracts from another.
10. **Platform access is never assignable from practice settings.** Feature flags, domains, secrets, and the practice list stay with Vet At Your Door.

6. What this changes in the product today

Facts about the current build, so the scope of the work is visible before you approve.

Today	What the approved matrix requires
Scout's role catalog contains Superadmin, Admin, Inventory Manager, Practice Manager, Technician, Client Liaison, Lead Client Liaison, Lead Technician. There is no provider role.	Add Veterinarian, Billing & AR, and Observer. Rename Admin to Practice Admin. Retire Superadmin from the practice catalog.
Practice roles currently control exactly one thing: which appointment types a person may book manually.	Roles become the source of truth for every row in Section 4. This is the bulk of the build.
Everything else is gated on the <i>login</i> role — <code>admin</code> , <code>superadmin</code> , <code>employee</code> — which is a separate system from practice roles.	The login role shrinks to an account type. Capability moves to practice roles so the two stop disagreeing.
A permission framework exists in the frontend (<code>canSeeRouting</code> , <code>canSeeDoctorDay</code> , <code>canManageUsers</code>) but nothing ever populates it, so those checks pass for everyone.	Either populate it from the practice roles or remove it. Right now it reads like protection that is not there.
Settings and Admin are all-or-nothing on login <code>admin</code> .	A Practice Manager can reach operational settings without also gaining the ability to create admin logins.
Inventory Manager cannot open suppliers, the count report, expiring, or cost reviews — those pages check for login	The role that is named for the job can actually do the job.

Today	What the approved matrix requires
admin .	
Square reconciliation is restricted to platform superadmin.	Opens to Practice Admin, Practice Manager, and Billing & AR.
Role and assignment endpoints are authenticated but not role-checked on the server.	Server-side enforcement, so permissions are not only a hidden button.
Imported eVet roles (Receptionist, KennelWorker, Groomer, Kiosk, BusinessOwner, and the rest) still appear in role data.	Hidden from the picker and remapped per Section 2.
There is no reason-code prompt on price changes, discounts, voids, or refunds, and no discount/void/refund audit report.	Needed for rows marked "reason code" and for the audit report in Section N. This is a meaningful piece of work and could be phased.

7. Decisions I need from you

1. **Discount cap.** What percentage can a DVM, technician, or liaison apply before it needs Practice Manager approval? The common setting is 10–15%.
2. **Billing & AR — keep it or fold it in?** If your liaisons already handle collections, delete the role and give Lead Client Liaison the refund, deposit, and reconciliation rows instead.
3. **Can a Practice Manager create staff logins?** The matrix says yes for non-admin logins. Say the word if that should be Practice Admin only.
4. **Voiding a deposit.** Currently Practice Admin and Practice Manager, with Billing & AR needing approval. Should Billing & AR be able to do it outright?
5. **Observer — needed?** Worth having for onboarding and for an outside bookkeeper, but it is a real role to build and test.
6. **Relief / locum DVMs.** Should they be a separate role (same clinical rights, no production reports, no access after their last shift), or just the DVM role deactivated when they leave?
7. **Branch scoping.** Inventory Manager and Practice Manager are branch-specific today. Should Lead Technician or Lead Client Liaison also be scoped per location?
8. **Who approves a lab result into the record?** The matrix says DVM only, with Lead Technician able to flag. Confirm that matches how your team actually works.
9. **Reason codes and the audit report.** Build them with the roles, or ship roles first and add the audit layer in a second phase?
10. **Anything a specific person needs** that the role they will hold does not allow. Easier to catch now than after rollout.

Approval — sign here and I will implement exactly what is above, including any edits you mark on the matrix.

 Approved by · Date

Draft generated from the Scout codebase (vayd-enterprise-manager and vayd-api). Nothing in this document has been implemented. Role names, slugs, and the current-state notes in Section 6 reflect the code as it stands today.